

February 23, 2026

Department of Health and Human Services

Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP)

Office of the National Coordinator for Health Information Technology (ONC)

VIA ELECTRONIC SUBMISSION – Federal Register

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Secretary Robert F. Kennedy, Jr., Department of Health and Human Services:

Siemens Healthineers appreciates the opportunity to discuss how the Department of Health and Human Services (HHS) can accelerate the adoption and use of artificial intelligence (AI) as part of clinical care. For over 20 years, Siemens Healthineers has been committed to advancing innovative AI clinical care and workflow solutions, to increase access and improve outcomes for all patients.

We are a team of 17,000 passionate employees across more than twenty-five locations in the United States, including R&D centers, manufacturing sites, and sales and service operations. At Siemens Healthineers USA, we pioneer breakthroughs in healthcare for everyone, everywhere. By bringing innovative technology to the United States market, we enable healthcare professionals to deliver high-quality care that changes lives. Our portfolio includes diagnostic and therapeutic imaging, radiotherapy treatment, laboratory diagnostics, molecular medicine, digital health, and enterprise services. Our solutions are integral to informed clinical decisions across the comprehensive healthcare landscape.

Our AI-enabled clinical applications and AI-powered solutions address major challenges that are facing the healthcare field. Siemens Healthineers' AI-Rad Companion Chest Computed Tomography (CT) provides otherwise unavailable quantitative lung lesion data which enables clinicians to make more informed clinical decisions. Additionally, developing solutions for managing increasing workloads allows clinicians to spend more time with patients, which helps to drive long-term success. As workloads are growing, diagnostics and treatments are becoming more complex; experts and physicians need a new set of tools that can handle large volumes of medical data quickly and accurately. Siemens Healthineers has developed a portfolio of AI-powered solutions that help advance more informed clinical decisions and automate and standardize workflows and complex diagnostics to meet the needs of individual patients.

For further information or to connect with Siemens Healthineers, please contact Leah Kegler (leah.kegler@siemens-healthineers.com) Head, Government Affairs, Americas.

Sincerely,

Specific Questions

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

At the heart of Siemens Healthineers three billion patient touchpoints, is a passion to drive better health outcomes for every patient, everywhere. We partner with leaders across the Administration, and throughout the health care ecosystem to improve primary care, advanced screening, and provide a multifaceted approach to chronic care for conditions like diabetes, cancer, and heart disease. A core component of this work is the integration of transformative innovations like artificial intelligence (AI) into clinical care. Without greater provider adoption and improved access, the opportunity it presents to change everyday health care challenges for patients will never be realized.

Centers for Medicare and Medicaid Services (CMS) reimbursement is a key driver of adoption of transformative innovation, like AI – especially in rural areas. While CMS has noted the potential of AI to increase access, improve outcomes, and lower costs, outdated laws and regulations are causing barriers that are hampering clinical adoption. The inconsistent, unreliable, and ad hoc approach to reimbursing software as a service (SaaS), and FDA approved AI-enabled medical devices, is having a crippling effect on provider adoption and patient utilization under Medicare. We provide specific recommendations below, that we believe will be a productive step towards bolstering adoption of innovative clinical AI tools in health care that can help us better address critical challenges that are resulting in poor health outcomes for patients.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care. Please provide specific changes and applicable Code of Federal Regulations citations.

As CMS stated in the CY2023 HOPPS/ASC Final Rule, “Novel and evolving technologies are introducing advances in treatment options that have the potential to increase access to care for Medicare beneficiaries, improve outcomes, and reduce overall costs to the program.” While CMS has recognized the value and the complex nature of algorithm-based services, machine learning, and clinical analytic software, the agency’s payment determinations have not been applied uniformly and consistently to guarantee appropriate payment levels for these products.

We suggest that CMS consider adopting the term algorithm-based healthcare services (ABHS), which include Software as a Service (SaaS) procedure. ABHS are clinical analytical services delivered by FDA-cleared devices to a healthcare practitioner that uses artificial intelligence, machine learning, or other similarly designed software to produce clinical outputs for the diagnosis or treatment of a patient's condition. ABHS provides quantitative and qualitative analyses, including new, additional clinical outputs that detect, analyze, or interpret data to improve screening, detection, diagnosis, and treatment of disease. Further, ABHS includes these services regardless of whether they are delivered by a third party or by a healthcare professional using AI software through the cloud, the web, a workstation, or as embedded in imaging acquisition hardware. Devices embedded with software may be preferable to certain providers based on their existing imaging equipment, workflow, and staffing requirements. Note that all procedures currently classified as SaaS by CMS should also be classified as ABHS.

CMS should utilize its SaaS policy for determining payment for ABHS procedures in the HOPPS, not its add-on code packaging policy. This approach will ensure that providers receive fair, appropriate, and stable compensation when they utilize ABHS to improve outcomes for Medicare beneficiaries. We also recommend that all new ABHS CPT codes be initially assigned to a New Technology APC (ambulatory payment classification) payment, based on manufacturer-supplied cost data, until sufficient claims data are collected to make a proper clinical APC assignment. It is important to acknowledge that current CMS practices already encompass this payment methodology and are not consistently used across all new CPT codes. Furthermore, these CPT codes should remain in the New Technology APC payment assignment for at least five years, which aligns with the lifespan of a Category III CPT code, and not be subject to the Universal Low Volume APC policy. This is necessary to ensure appropriate data collection and analysis can occur while hospitals adopt ABHS.

We request that CMS clarify and codify the SaaS payment policy as follows:

- CMS should adopt the term “algorithm-based healthcare services” (ABHS), which include Software as a Service (SaaS) procedure.
- CMS should formalize the exception to the packaged payment policy in regulatory text, including Software as a Service (SaaS) procedure.
- CMS should ensure ABHS that secure a separate and distinct CPT code through the AMA CPT coding process are exempt from CMS's add-on code packing policy and receive separate payment. CMS should initially assign ABHS CPT codes to a

New Technology APC payment for 5 years, based on manufacture-supplied cost data, until sufficient claims data is collected to make a proper clinical APC assignment. Additionally, ABHS APC assignments should not be subject to the Universal Low Volume APC Policy.

We offer the following recommended revision at 42 CFR 419.2 to formalize and codify the SaaS policy:

419.2 Basis of payment

(b) Determination of outpatient prospective payment rates: Packaged costs.

(18) Certain services described by add-on codes except as provided in § 419.2(d).

(d) Determination of hospital outpatient prospective payment rates: Separately paid costs. Software as a Service procedures assigned to CPT add-on codes will be paid separately at an amount equal to the amount of the payment for the add-on procedure when the service is furnished without the standalone CPT code. These codes will be assigned to identical ambulatory payment classifications and status indicator assignments as their standalone codes. Separate payment will be made available for these services if the following criteria are met:

(1) The service has been approved by the FDA and has received a CPT code;
and

(2) The service is billed concurrently with the associated service code.